

Lacrimal localisation bypass and failed drainage

Part 2 | Tearing: separate production from drainage

A patent duct does not establish normal tear clearance.

Finding	Implication
Surface irritation	Reflex tear production can increase
Punctum or lid malposition	Entry or pump component
Canalicular obstruction	Proximal mechanical resistance
Distal duct obstruction	Sac-to-nose drainage problem

Examine the ocular surface, tear lake, punctal position and blink before reaching for a probe. The useful question is where tears fail to enter or leave the system. More than one component can coexist, so a positive syringing result should be integrated with symptoms and examination rather than treated as a complete physiological test.

Soft and hard stops localise different structures

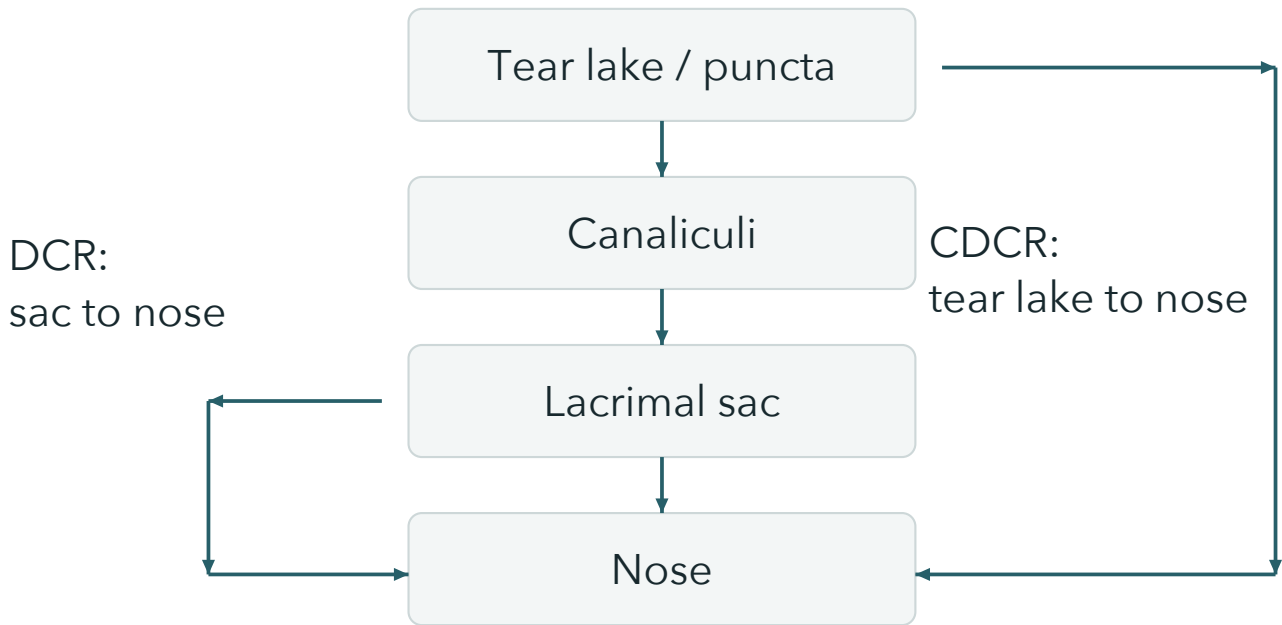
Avoid forcing a probe through a presumed obstruction.

Finding	Implication
Soft stop	Probe has not reached the medial sac wall
Hard stop	Canalicular passage to the sac is established
Same-punctum reflux	Supports obstruction in the cannulated pathway
Opposite-punctum reflux	Interpret with canalicular and distal anatomy

Record which punctum was cannulated, where resistance occurred and the direction of reflux. These observations are more useful than a global blocked-system label. Gentle technique protects the remaining passage. In an examination case, draw the route and show why the proposed level fits the stop and reflux pattern, then state what further assessment is needed.

DCR and CDCR bypass different levels

An operation below the obstruction will not restore the missing route.



Original pathways: DCR from sac to nose; CDCR from tear lake to nose.

DCR requires a usable route into the lacrimal sac. Severe unreconstructable canalicular obstruction may need a conjunctivodacryocystorhinostomy with a Jones tube, bypassing the proximal system. Explain the continuing tube-care burden and potential displacement or blockage. Choose the operation from the anatomical level and patient capacity for follow-up, not from epiphora severity alone.

A patent DCR with continuing epiphora

Investigate function and alternative causes before enlarging the ostium.

- Confirm anatomy and nasal ostium findings
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- Reassess surface, puncta, lid and blink
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- Consider physiological drainage assessment where useful
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- Target the remaining mechanism rather than repeat the label

Free syringing establishes passage under pressure, not normal clearance with a blink. Pump weakness, malposition or excess tear production can explain persistent symptoms. Compare the original indication with current anatomy and function. A further operation should have a specific target; technical ostium success alone cannot guarantee patient-reported relief.

Counselling about a bypass

Success includes manageable aftercare and useful symptom relief.

Finding	Implication
DCR	Assess ostium, infection and symptom response
Jones tube	Plan cleaning, position checks and long-term care
Persistent tearing	Distinguish blockage from functional insufficiency
New atypical signs	Reassess the underlying diagnosis

Explain what the bypass changes and what it leaves unchanged. A tube can create a drainage route without restoring a normal pump, and maintenance can be substantial. Agree which outcome the patient values and how failure will be investigated. This makes the treatment choice more realistic than quoting a success percentage without its definition.